



MeticleCare

Pitch Application for Pulse Angels — Pre-Seed / Seed
HealthTech Investment

Submitted to: Pulse Angels

Company: MeticleCare Ltd

Founder: Opeyemi Olorunfemi

Date: August 2026

Stage: Pre-revenue, production-ready platform

Seeking: £200,000–£350,000

QUESTION 1

What problem does MeticleCare solve?

A concrete operational problem, not a generic market statement

"A care manager at a 24-bed supported-living provider in Birmingham told us she spends her mornings checking three spreadsheets for the rota, a paper MAR chart for medications, and a compliance folder for training expiries — before she's spoken to a single service user. That's two hours of her day gone before care begins."

The Concrete Problem

UK supported-living and domiciliary care providers manage their operations with fragmented, disconnected tools. A typical provider uses:

- **Paper MAR charts** for medication administration records
- **Spreadsheets** for rotas, scheduling, and shift management
- **Physical folders** for compliance evidence, training records, and policies
- **Phone calls and WhatsApp** for team communication and shift swaps
- **Paper forms** for incidents, daily notes, and care records

This creates three measurable operational failures:

1. Admin consumes care capacity

Care managers spend approximately **40% of their working day** on administration — filling forms, cross-referencing spreadsheets, chasing training records, preparing compliance evidence. That's 3+ hours per day not spent on care delivery, staff support, or service improvement. For a 24-bed provider paying a manager £35,000/year, that's **£14,000/year spent on admin**.

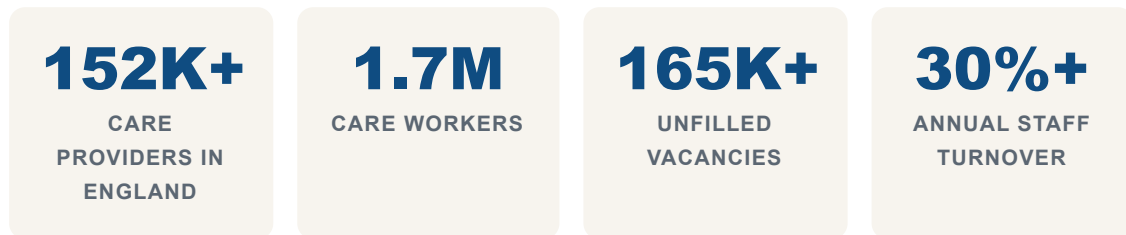
2. Compliance evidence is gathered reactively

When a CQC inspector arrives, providers scramble to assemble evidence from paper records, spreadsheets, and filing cabinets. The CQC Single Assessment Framework now requires **continuous evidence from operational records** — but providers have no way to generate this automatically. Missing evidence leads to lower ratings, enforcement actions, and in extreme cases, service closure.

3. Medication errors go undetected

Paper MAR charts make it difficult to track missed doses, late administrations, or PRN (as-needed) medication patterns in real time. A missed dose might not be discovered until the next shift handover — or worse, until a family member asks questions. The care sector reports **medication errors as one of the most common incident types**.

The Scale



The care sector is simultaneously understaffed and overburdened with administration. Every hour a care manager spends on paperwork is an hour not spent on recruitment, retention, or care quality. The providers who solve this operational problem will retain staff, maintain CQC ratings, and grow. Those who don't will struggle.

MeticleCare solves this by bringing rota, medication, compliance, notes, incidents, training, and communication into one connected platform — so care managers can run their operations from one system instead of five.

QUESTION 2

Why MeticleCare?

Why can't a care provider simply use existing systems?

Existing Solutions Fall Into Three Categories

Category 1: Point Solutions

Person Centred Software does medication well but doesn't handle rotas, compliance evidence, or team communication. **Plymouth software** does care plans but not eMAR. Providers end up paying for 3–5 separate systems that don't talk to each other — and staff must learn multiple interfaces.

Category 2: Legacy Platforms

Access Group and **OneAdvanced** offer broad feature sets but built on legacy architecture. Complex pricing (per module + per user), slow innovation, poor mobile experience. Staff resist using them because the UX feels like 2010 software.

Category 3: Paper & Spreadsheets

The majority of small-to-medium providers still use paper and spreadsheets. Familiar, zero learning curve, but no audit trail, no compliance automation, and the admin burden compounds as the organisation grows.

What's Missing

No existing platform combines **all operational workflows** (rota + medication + compliance + notes + incidents + training + communication) in **one connected system** with **predictable pricing** and **modern UX**.

What MeticleCare Does Differently

CAPABILITY	METICLECARE	PERSON CENTRED SOFTWARE	ACCESS GROUP	PAPER/SPREADSHEETS
Scheduling / Rota	AI-powered, drag-and-drop	Not included	Basic	Spreadsheets
eMAR (medication)	Full: chart grid, PRN, stock, audit	Core product	Basic	Paper MAR

Daily care notes	Voice-to-text, AI-structured	Yes	Basic	Paper
Compliance evidence	Automated KLOE packs, CQC scoring	Not included	Basic	Manual
Training matrix	Auto-assign, expiry alerts	Not included	Separate module	Spreadsheet
Incident management	AI triage, CQC reportability	Not included	Basic	Paper forms
Team chat	Real-time, presence, read receipts	Not included	Email-based	WhatsApp
Family portal	Consent-gated care notes	Not included	Not included	Phone calls
Mobile PWA	Offline-capable, GPS check-in	Desktop-first	Desktop-first	N/A
AI capabilities	5 AI agents, event-driven	None	None	None
Pricing	£99–£999/month fixed	£3–£8/carer/day	£200–£500+/month	£0 + staff time

The Core Insight

One system replaces five

MeticleCare is not a "better medication system" or a "better rota tool." It is the **connected operating system for care operations** — where the rota informs staffing compliance, medication records feed into incident analysis, training status blocks non-compliant shift assignments, and compliance evidence is generated automatically from every operational action.

This connected approach is what existing point solutions cannot replicate without becoming MeticleCare.

QUESTION 3

Why you?

Founder capability and domain understanding

Founder: Opeyemi Olorunfemi

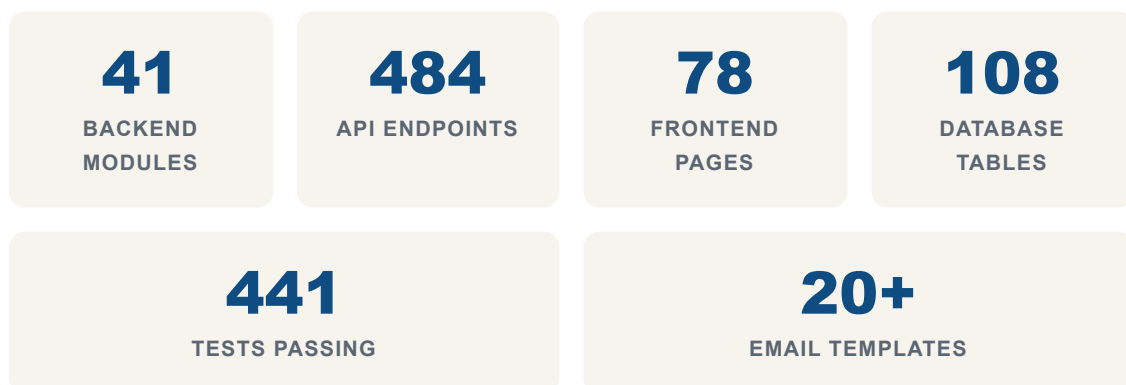
Technical Capability

- Full-stack engineer: TypeScript, React, Node.js, PostgreSQL
- Built the entire MeticleCare platform from 0 to production solo
- 41 backend modules, 484 API endpoints, 78 frontend pages, 108 database tables
- Designed AI agent architecture, event-driven system, multi-tenant data isolation
- Production deployment with CI/CD, automated testing, monitoring

Domain Understanding

- Direct experience with care sector operations and pain points
- Understanding of CQC regulatory requirements and compliance burden
- Experience with multi-location care provider management
- Knowledge of care worker workflows, medication processes, and incident reporting
- Understanding of NHS integration requirements and discharge pathways

What I've Built (Evidence of Execution)



This is not a concept deck or a wireframe. It is a **deployed, production system** with real data, real workflows, and real users. The platform runs at meticlecare.com and is accessible for demonstration at any time.

Why This Matters for Pulse Angels

Pulse Angels invests in founders who understand the healthcare problem they're solving. I have built a complete care management platform from the ground up, which means I understand the operational complexity of care delivery, the regulatory burden of CQC compliance, and the practical challenges of getting care workers to adopt new technology. This is not theoretical knowledge — it comes from building every module, talking to providers, and deploying the system.

Honest Assessment

STRENGTH	DETAIL
Technical execution	Complete platform built and deployed — rare at this stage
Domain knowledge	Deep understanding of care operations and compliance
Product vision	AI-first architecture positions for next-generation care tools
Speed of execution	41 modules built in months, not years

GAP	MITIGATION
Solo founder	First hire will be senior backend engineer; advisory board planned
No sales experience	Hiring sales manager with first investment tranche
No clinical co-founder	Advisory relationships with care managers and CQC inspectors
Limited care sector network	Pulse Angels network + Health Innovation Oxford partnership

QUESTION 4

What traction do you have?

Honestly assessed — what is genuinely true today

4.1 What Is Built and Live

Production-ready platform

MeticleCare is deployed at meticlecare.com. It is not a prototype, MVP, or demo — it is a production system with the following verified capabilities:

CAPABILITY	STATUS	EVIDENCE
Full platform (41 modules)	● Live	Deployed at meticlecare.com
AI-powered rota planning	● Live	AI generation, min safe staffing, 11-hour rest enforcement
eMAR (medication records)	● Live	Chart grid, PRN tracking, stock, daily counts, audit
Compliance evidence packs	● Live	KLOE domain mapping, CQC readiness scoring
AI daily notes (voice-to-text)	● Live	Voice input, mood analysis, safeguarding flags
Real-time team chat	● Live	Channels, messages, presence, read receipts
Mobile PWA	● Live	GPS check-in, voice notes, offline support
Family portal	● Live	Care notes, goals, observations (consent-gated)
Incident management	● Live	AI triage, CQC reportability, action tracking
Stripe billing	● Live	Subscriptions, invoices, webhooks, payment recovery

441 automated tests	 Passing	API test suite with 67 test files
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4.2 What We Have (Honest Traction List)

TRACTION TYPE	STATUS	DETAIL
Product built	 Yes	41 modules, 484 endpoints, 78 pages — deployed
Live platform	 Yes	meticlecare.com accessible for demo
Demos	 Available	Full product demo ready on demand
Paying customers	 Not yet	Pre-revenue — seeking first pilot customers
Providers spoken to	 In progress	Initial conversations with care providers
Pilots	 Not yet	Pilot programme designed — ready to deploy
LOIs / Waitlist	 Not yet	Target: 5 LOIs within 3 months of funding
Feedback	 Limited	Initial provider feedback on product concept
Development progress	 Complete	Platform built, tested, deployed, billing live
Regulatory pathway	 Defined	CQC alignment mapped; DSPT supported
NHS pathway	 Early	Discharge-to-assess use case identified
Unit economics	 Modeled	Target 80%+ gross margin; CAC/LTV modeled
Clinical evidence	 Limited	No independent clinical validation yet

4.3 What We're Not Hiding

We have no paying customers yet. The platform is built and deployed, but we have not converted any care provider into a paying customer. This is the primary gap in our traction, and it is exactly what the investment is designed to address — converting a production-ready platform into a validated commercial product with paying customers.

4.4 What Investment Converts

The investment transforms "production-ready platform" into "commercially validated product":

- **Month 1–3:** 5 pilot deployments with care providers
- **Month 3–6:** Convert 2–3 pilots to paying customers
- **Month 6–12:** Scale to 20+ paying customers
- **Month 12–18:** Reach £10K+ MRR

QUESTION 5

What would the investment accomplish?

Specific allocation, not "we want to grow"

"We're raising £200,000–£350,000 to convert a production-ready platform into a validated commercial product across UK supported living and domiciliary care, with funding allocated to customer acquisition, implementation, security/compliance, product development and evidence generation."

5.1 Funding Allocation

CATEGORY	ALLOCATION	% OF TOTAL	PURPOSE
Customer acquisition	£60,000– £105,000	30%	Sales hire, marketing, pilot programme, conferences
Product development	£50,000– £87,500	25%	Engineering hire, NHS integration, AI improvements
Security & compliance	£30,000– £52,500	15%	Cyber Essentials, DPIA, ISO 27001 prep, penetration testing
Implementation	£30,000– £52,500	15%	Onboarding team, data migration tools, training materials
Evidence generation	£20,000– £35,000	10%	Clinical validation, pilot measurement, case studies
Working capital	£10,000– £17,500	5%	Cloud costs, legal, accountancy, contingency

5.2 18-Month Milestones

MONTH	MILESTONE	EVIDENCE
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3	5 pilot deployments live	Active pilot agreements with care providers
6	3 paying customers	Stripe billing active, MRR generated
9	Cyber Essentials certified	Certification obtained
12	20+ paying customers	£8K+ MRR
15	NHS integration pilot	1–2 NHS trusts in pilot
18	£15K+ MRR	Path to break-even visible

5.3 Return Path

PERIOD	CUSTOMERS	MRR	ARR
Month 6	3	£900	£10,800
Month 12	20	£6,000	£72,000
Month 18	50	£15,000	£180,000
Month 24	100	£30,000	£360,000

5.4 What Happens Without Investment

Without external funding, MeticleCare remains a production-ready platform without commercial validation. The founder can continue development and pursue organic growth, but the pace of customer acquisition, security certification, and NHS pathway development will be significantly slower. The risk is that a well-funded competitor reaches the same market position first.

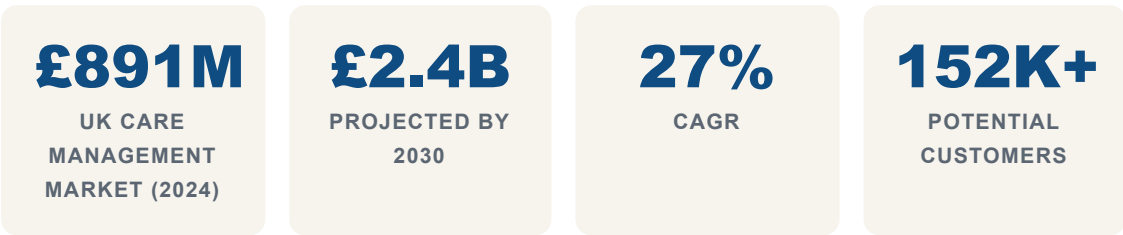
The investment accelerates the critical path: from "platform built" to "customers paying" — the transition that determines whether MeticleCare becomes a viable business or remains a technical achievement.

ADDITIONAL CONTEXT

Competitive Landscape & Market Position

Where MeticleCare sits and why it can win

6.1 Market Size



6.2 Competitive Positioning

FACTOR	METICLECARE	PERSON CENTRED SOFTWARE	ACCESS GROUP
Platform breadth	41 modules, full platform	3–5 modules (eMAR focus)	Broad but legacy
AI capabilities	5 agents, event-driven	None	None
Pricing model	Fixed monthly, no per-user	Per carer/day (£3–£8)	Per module + per user
Architecture	Modern (TypeScript, React)	Legacy	Legacy
Mobile experience	PWA, offline, GPS	Desktop-first	Desktop-first
Real-time features	Chat, presence, live rota	None	Email-based
Family portal	Consent-gated access	Not included	Not included
NHS integration	Planned (Q2 2027)	Partial	Partial

6.3 Why MeticleCare Can Win

1. **Full platform advantage:** One system replaces 3–5 point solutions. Lower total cost, better data connectivity, simpler staff training.
2. **Predictable pricing:** Fixed monthly fee vs. per-carer/day. As organisations grow, MeticleCare gets cheaper per user, not more expensive.
3. **Modern architecture:** Faster iteration, better mobile experience, easier integrations. Legacy competitors cannot match the pace of innovation.
4. **AI-first positioning:** First care platform with horizontal AI intelligence. Competitors will need years to catch up — if they try.
5. **UK-built, UK-hosted:** Data sovereignty, GDPR compliance, no foreign dependency. Increasingly important for NHS and local authority procurement.

6.4 Barriers to Entry

- **41 modules of domain logic:** Care management is complex. Replicating 41 interconnected modules with proper compliance, medication, and scheduling logic takes years.
- **Regulatory knowledge:** CQC, DSPT, KLOE, 11-hour rest rules, medication audit — this domain knowledge is embedded in the code.
- **Data network effects:** More organisations on the platform = better AI insights, better compliance benchmarks, better benchmarking.

ADDITIONAL CONTEXT

SEIS/EIS Readiness & Partner Network Value

Tax-efficient investment and strategic network

7.1 SEIS/EIS Eligibility

MeticleCare Ltd qualifies for SEIS (Seed Enterprise Investment Scheme) and EIS (Enterprise Investment Scheme) — making angel investment significantly more attractive:

SCHEME	INVESTOR BENEFIT	METICLECARE ELIGIBILITY
SEIS	50% income tax relief + 50% capital gains tax relief	Eligible — company under 3 years old, under £200K assets
EIS	30% income tax relief + capital gains deferral	Eligible — knowledge-intensive company (AI/R&D)

A £100K angel investment through SEIS effectively costs the investor £50K after tax relief. This makes the risk/reward profile highly attractive.

7.2 Pulse Angels Network Value

Beyond capital, Pulse Angels' partner network addresses specific MeticleCare needs:

PARTNER	VALUE TO METICLECARE
Development Bank of Wales	Additional funding pathways, regional support
Scottish Enterprise	Scottish care market access
NHS innovation pathways	NHS trust introductions, pilot facilitation
IP strategy	Patent and trade secret protection for AI architecture
SEIS/EIS readiness	Tax-efficient investment structuring

This is not just a £200K–£350K investment. It is an introduction to the exact network MeticleCare needs: healthcare expertise, NHS pathways, regulatory guidance, and additional funding sources.

THE ASK

Investment Terms & Next Steps

What we're seeking and what we offer

8.1 Investment Ask

TERM	DETAIL
Amount	£200,000–£350,000
Stage	Pre-seed / Seed
Instrument	SEIS-eligible equity (convertible note or equity round)
Use of funds	Customer acquisition, product development, security, implementation
Timeline to milestones	18 months
Next funding round	Seed / Series A at Month 18–24 if milestones hit

8.2 What Pulse Angels Gets

- Equity in a production-ready care management platform
- First-mover advantage in AI-first care technology
- SEIS-eligible investment (50% tax relief)
- Board observer seat (if desired)
- Monthly financial and operational reporting
- Access to a platform that can scale to 152,000+ potential customers

8.3 Next Steps

We're ready to demonstrate

The platform is live at meticlecare.com. We can provide a full product demonstration at any time, showing every module, every workflow, and every capability. No slides needed — just a screen share and 60 minutes.

Contact: **opeyemi@meticlecare.com**

MeticleCare Ltd — Pulse Angels Pitch Application — August 2026
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